

Accidental Tracheostomy Dislodgement Algorithm

Desaturation
Think : DOPE

Tracheostomy tube visibly out or suspected out

Is Child Breathing?

YES
Breathing

- ☐ Apply Oxygen to mouth and Stoma.
- ☐ Replace with new tracheostomy if able and if the stoma is mature
- ☐ Continue monitoring ABCDE

No Breathing or Gasping

- ☐ Start CPR
- ☐ Activate Arrest Code
- ☐ Call PICU, Anaesthesia, Pediatric surgical on call
- ☐ Anticipate difficult airway

- ☐ Check Patency of tracheostomy
- ☐ Remove HME (Heat moisture exchange) and other attachments.

Able to Pass Suction Catheter through Tracheostomy?

- ☐ Yes, able to pass suction catheter (Partially Blocked Tracheostomy)
- ☐ Co2 detector Present
- ☐ Chest Rise present on bagging
- ☐ Continue monitoring ABCDE

- ☐ NO, Not able to Pass suction catheter/ Or Tracheostomy visibly out
- ☐ No End Tidal Co2 Trace on Co2 detector
- ☐ No Chest Rise on Bagging

- ☐ Bag with 2 persons technique and close Stoma with gauze
 - ☐ Replace Tracheostomy if stoma is mature > 7 days and if competent
- OR
- ☐ Intubate orally if unable to replace tracheostomy

Causes of Desaturation in a child with tracheostomy "THINK DOPE"

D: Displacement of tracheostomy tube
O: obstruction of tracheostomy tube
P: Pneumothorax
E: Equipment failure

Risk Factors for Tracheostomy

Dislodgement:

- ☐ Loose tapes
- ☐ Short neck
- ☐ Altered mental status (agitation, inadequate sedation)
- ☐ Incorrect Tube length
- ☐ Tension on the tracheostomy tube

If Child de saturates, Think DOPE:

- ☐ Displacement of tube (Check with Co2 detector)
- ☐ Obstruction of tube : (Check if able to Pass Suction catheter)
- ☐ Patient : pneumothorax, pulmonary edema, atelectasis, bronchospasm (Obtain CXR)
- ☐ Equipment failure (Hand bag with 100% oxygen)

Required equipment at the Bed side

- ☐ Self Inflating Bag (Ambu bag) and face mask with O2 connection
- ☐ Tracheostomy same size and size smaller
- ☐ End-Tidal Co2 detector
- ☐ Tracheal Dilator
- ☐ Suction catheter
- ☐ Tracheostomy tie and scissor